

Urethral Stricture (Female)

Patient Information · A scar that narrows the urethra and slows the stream

WARWIKI

The urethra is the short tube that carries urine out of the bladder. In a urethral stricture, **scar tissue narrows it**. In women it is uncommon, and its symptoms look like other bladder problems, so it can take years to find. Stretching often helps only for a while. **Urethroplasty** (repair surgery) gives the longest-lasting result.

Signs of a Stricture

- A **weak or slow stream**, straining, or a feeling that you have not emptied
- Going **often or urgently**, and getting up at night to urinate
- **Bladder infections** that keep coming back
- At times, not being able to urinate at all

These symptoms are often blamed on an **overactive bladder**. Ask about a stricture if your symptoms do not improve with the usual treatments.

Benefits and Risks in Numbers

about
Half

stayed open about 3 years after stretching (dilation) alone

more than
8 in 10

had a lasting good result after urethroplasty, in pooled studies

fewer than
1 in 10

had new leaking of urine after the repair, in most studies

Numbers are averages from published studies. Your own risk may differ; ask your team.

How It Is Found

- An **exam** looks at the urine opening, the vaginal tissue and any skin condition.
- A **flow test** measures your stream, and a scan checks how well you empty. A **urine test** looks for infection.
- **Calibration**: small smooth tubes are gently passed to measure the width. A tube that will not pass is a clue, not proof.
- **Cystoscopy**: a thin camera looks at the narrow part.
- An **X-ray while you urinate**, often with bladder pressure tests (**video urodynamics**), shows where the narrowing is and how long it is. With the camera look, it helps tell a scar apart from a bladder or muscle problem.

Learn the Terms

Urethra

The tube that carries urine from the bladder out of the body.

Stricture

A scarred, narrow part of the urethra.

Dilation

Gently stretching the narrow part with smooth rods or a balloon.

Self-dilation

Passing a smooth catheter yourself on a schedule to keep the urethra open.

Urethroplasty

Surgery that widens the urethra with a patch of tissue.

Graft

A patch of your own tissue, often from inside the cheek, moved to the urethra.

Flap

Nearby vaginal or labial skin moved over with its own blood supply.

Video urodynamics

Bladder pressure tests done with X-ray pictures.

What Causes It

Often no cause is found. Known causes include past catheters, stretching or surgery on the urethra (for example for a diverticulum, a fistula or leaking), a pelvic injury, radiation, thin tissue after menopause and the skin condition **lichen sclerosus**.

Which treatment is right for me? Stretching is often tried first. If the narrowing comes back, you can repeat the stretch and then keep it open with self-dilation, or have urethroplasty. Guidelines advise urethroplasty when it comes back a second time and self-dilation does not suit you, or any time you want the most lasting fix.

Dilation	Self-dilation	Urethroplasty	After that
Done in the office or under anesthesia.	Ongoing, as often as your team advises.	Catheter for about 1–3 weeks, often with an X-ray before it comes out.	Visits to check your stream and emptying.

Your Choices

- **Dilation.** Quick, but the scar often narrows again, and repeated stretching alone seldom lasts.
- **Self-dilation.** Keeps the urethra open but does not cure the scar. Your team teaches you how.
- **Urethroplasty.** Done through the vagina. The narrow part is opened and widened with a **graft** from inside your cheek or a **flap** of nearby vaginal or labial skin. With lichen sclerosus, cheek tissue is usually used. Surgery is often planned about 3 months after the last stretch.

Tell your team if you have had a **mesh sling**. Stretching is usually avoided after one.

Recovery After Urethroplasty

- You go home with a **catheter**, and sometimes a second tube through the lower belly. Do not pull on them.
- Spotting from the vagina for 1–2 weeks is common. The stitches dissolve.
- Put nothing in the vagina, and avoid sex, until your team says it is safe.
- Walk every day. Bladder cramps (spasms) are common while the catheter is in; ask about medicine.
- With a cheek graft, your mouth is sore for 1–2 weeks. It has its own handout.
- A tight stricture can help hold urine in. Opening it can **show leaking** that needs its own treatment, so your team checks your bladder control before surgery.

Call your care team if you have:

- A temperature of **100.4 °F (38 °C)** or higher, or chills
- Burning, pain or swelling that gets worse instead of better
- A catheter that falls out. Do not put it back in yourself.
- A stream that is slowing again

Go to an emergency department if:

- The catheter stops draining, or you cannot urinate, and your bladder feels full or painful
- Heavy bleeding from the vagina, or urine like red wine or with clots
- You cannot reach the office and are worried

Three Things to Remember

1. A stricture in women is uncommon and easy to mistake for other bladder problems.
2. Stretching often helps only for a while. Urethroplasty lasts longest.
3. Tell your team if your stream slows again, even years later.